



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Inpatient Transfers from ICU to Mental Health Inpatient Unit

Reference #:FSFH-HW-POL-0104

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service Intensive Care Unit	Medical, Nursing

1. Introduction

This document outlines the process for the safe transfer of a patient from the Fiona Stanley Hospital (FSH) Intensive Care Unit (ICU) post extubation to a mental health inpatient ward. It includes the requirements for ascertaining readiness of the patient for transfer and consideration of the limitations of the mental health inpatient wards to provide acute medical care e.g., no piped gases at the bedside.

2. Terminology

Extubation	Removing of an endotracheal tube (ETT) which is the last step in removing a patient from a mechanical ventilator.
Extubation failure	The need to be reintubated within 48 hours of extubation. The percentage of patients intubated for management of a mental health diagnosis at FSH who require reintubation is low (~1.2%).
Adult Observation and response chart (AORC)	A physical observation chart used for patients aged 16 years and over. Includes early warning indicators to assist staff to recognise and respond appropriately to clinical deterioration.
Adult deterioration detection System (ADDS)	System within the AORC used to score physiological observations in patients 16 years and over. Scores are added together to identify patient deterioration and the next steps to take.
Personal support person	Any of the following people supporting someone experiencing a mental illness: close family member, carer, nominated person, parent or guardian of a child, guardian of an adult.

The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

3. Policy

To optimise patient safety during transfer, patients may only be transferred from ICU to a mental health inpatient bed between the hours of 08:00-18:00hrs where:

- the patient has been extubated for four hours or more
- the patient is haemodynamically stable and not requiring supplementary oxygen therapy.

Before transferring a patient from the ICU to a mental health ward, ensure the patient is

- tolerating oral intake
- passing urine (urinary indwelling catheter is to be removed and trial void needs to be successfully completed)
- not receiving intravenous therapy or treatment

4. Procedure

4.1. Patient movement out of ICU

- ICU nursing staff place the patient on Enterprise Bed Management (EBM) for a ward transfer
- The receiving treating team consultant and ward Nurse Unit Manager (NUM) /delegate or Hospital Out of Hours (HOOT) on weekends will review and discuss the patient's clinical suitability.
- The receiving NUM/delegate will confirm acceptance and bed availability
- Bed readiness and timing of transfer is confirmed by the receiving ward and information updated on EBM
- ICU Shift Coordinator phones the relevant ward coordinator to hand over all specific patient information
- The Shift Coordinator /ICU Pod Leader will ring the ward shift coordinator immediately prior to transfer of the patient
- The patient will be transferred with a Nursing Transfer Summary documenting the patient's observation monitoring plan for the following 24 hours as per the ICU treating team. The Nursing Transfer Summary will include a clearly documented management plan for any unresolved medical issues

4.2. Patient arrival mental health ward

- A full set of physiological observations must be documented following transfer to the receiving ward and then as per the patient's ICU observation monitoring plan
- The AORC will be used to detect deterioration and guide the required level of response and care escalation based on the patient's physiological observations and ADDS score
- Ensure the patients personal support person is informed of transfer of care to the mental health unit
- Unless contraindicated encourage:
 - mobilization
 - deep breathing and coughing
 - continued diet and fluids
- Request review by physiotherapy if indicated

5. Compliance Monitoring

The Nurse Unit Manager of the receiving ward will evaluate compliance with this policy and procedure via clinical incident review and monitoring.

6. Related Policy Documents

FSFHG:

- Recognising and responding to acute deterioration (FSFH-HW-POL-0033)
- Observation and response chart (FSFH-HW-POL-0032)
- Intensive care unit patient standard of care (FSFH-ICU-POL-0001)

7. Related Standards

NSQHS Standards:

- Clinical governance
- Comprehensive care
- Communicating for safety
- Recognising and responding to acute deterioration

Chief Psychiatrists Standards for Clinical Care:

- Care Planning
- Physical health care of mental health consumers
- Transfer of Care

8. Related legislation

[Mental Health Act 2014](#)

9. Authorisation

EXECUTIVE SPONSOR: Service 5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2023	S5 Nurse Director	S5 SQR committee NMPGC	Policy and Practice Committee	10/2027